



SURGICAL PATHOLOGY REPORT

NAME:	SURG PATH #:
MR #:	SPECIMEN CLASS:
BILLING #:	ALT ID #:
LOCATION:	DATE OF PROCEDURE:
AGE:	DATE RECEIVED:
DOB:	TIME RECEIVED:
PHYSICIAN:	DATE OF REPORT:
COPY TO:	DATE OF PRINTING:

Material Received:

- A: left breast SLN #1 time out
- B: left breast SLN #2 time out
- C: left breast SLN #3 time out
- D: left breast SLN #4 time out
- E: left breast lumpectomy time out
- F: additional anterior left breast tissue

ICD-O-3
Carcinoma, infiltrating lobular NOS 8520/3
Site: ^{cell} ① Breast NOS C50.9
^{path} ② Breast, upper-outer quadrant C50.4
12/5/2014

History:

year-old female with history of invasive lobular carcinoma of the breast.

Final Diagnosis:

- A. Lymph node (1), "left breast SLN #1", biopsy:
There is no evidence of malignancy in one lymph node. (0/1)
Deeper sections and a pancytokeratin immunostain are negative in support of the above diagnosis.
- B. Lymph node (1), "left breast SLN #2", biopsy:
There is no evidence of malignancy in one lymph node. (0/1)
Deeper sections and a pancytokeratin immunostain are negative in support of the above diagnosis.
- C. Lymph node (1), "left breast SLN #3", biopsy:
There is no evidence of malignancy in one lymph node. (0/1)
Deeper sections and a pancytokeratin immunostain are negative in support of the above diagnosis.
- D. Lymph node (1), "left breast SLN #4", biopsy:
There is no evidence of malignancy in one lymph node. (0/1)
Deeper sections and a pancytokeratin immunostain are negative in support of the above diagnosis.
- E. Breast, "left breast lumpectomy", lumpectomy:
Invasive lobular carcinoma, moderately differentiated. See comment.
Lobular carcinoma in-situ and atypical lobular hyperplasia.
Previous biopsy site changes.
- F. Breast, "additional anterior left breast tissue", excision:
Microscopic focus of invasive lobular carcinoma. See comment.
Lobular carcinoma in-situ and atypical lobular hyperplasia.

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Order Number:

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Comment:

INVASIVE CARCINOMA OF THE BREAST

Specimen Type: Lumpectomy

Laterality: Left

Tumor Site: 1:00, 6 cm FTN

Histologic Type: Invasive lobular carcinoma

Size of Invasive Component: 1.5 cm in greatest dimension in the main lumpectomy. 0.3 cm in greatest dimension in the separately submitted "additional anterior left breast tissue".

Tumor Multicentricity: Yes

Surgical Margins: The carcinoma is focally 1.5 mm from the anterior margin and focally 1.75 mm from the inferior margin in the main lumpectomy (Specimen E); all other margins are greater than 0.2 cm away. In specimen F ("additional anterior left breast tissue") which is unoriented, carcinoma is present at one shave margin/tissue edge while all other margins are 0.2 cm or greater away.

Histologic Grade (Nottingham Histologic Score): II/III

Tubule Formation: 3

Nuclear Grade: 2

Mitotic Count (40x objective): 1

Total Nottingham Score: 6/9

Ductal Carcinoma In-situ (DCIS): Absent

Lobular Carcinoma In-situ (LCIS): Present

Lymph-Vascular Invasion: Not identified

Perineural Invasion: Not identified

Tumor Necrosis: Absent

Nipple Involvement: Not applicable

Skin Involvement: Not applicable

Lymph Node Sampling:

Sentinel lymph node(s) only

Total number of involved nodes/total nodes found: 0/4

Non-neoplastic Breast Tissue: Focal atypical ductal hyperplasia. Fibrosis and prior biopsy site changes.

Prognostic markers: Ordered on prior biopsy. See for addendum

Time between tumor removal and placement into formalin $\leq$ 1 hour: Yes

Fixation Time between 6-48 hours: Yes

Pathologic Staging:

pT1c snN0(i-)

Primary Tumor (Invasive Carcinoma) (pT)

pT1c: Tumor >10 mm but $\leq$ 20 mm in greatest dimension

Regional Lymph Nodes (pN)

Modifier (required only if applicable)

(sn): Only sentinel node(s) evaluated. If 6 or more sentinel nodes and/or nonsentinel nodes are removed, this modifier should not be used.

pN0 (i-): No regional lymph node metastases histologically, negative IHC

Distant Metastasis (M)

Not applicable

The pathologic stage assigned here should be regarded as provisional, as it reflects only current pathologic data and does not incorporate full knowledge of the patient's clinical status and/or prior pathology.

In Specimen F, immunohistochemical stain for smooth muscle myosin supports the diagnosis.

Attestation:

By this signature, I attest that I have personally formulated the final interpretation expressed in this report and that the above diagnosis is based upon my examination of the slides and/or other material indicated in this report.

Electronically Signed Out By

Interpreted by:

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Resident

Gross Description:

A. Received in formalin labeled with the patient's name "left female breast sentinel lymph node #1 out at " are two tan-pink lymph nodes measuring 0.9 x 0.9 x 0.6 cm and 1.4 x 1.3 x 0.6 cm. The smaller lymph node is inked black. Both lymph nodes are serially sectioned and submitted entirely in cassette A1.

B. Received in formalin labeled with the patient's name "female left breast sentinel lymph node #2 time out " is a 2.3 x 1.8 x 0.6 cm aggregate of yellow-tan fibroadipose tissue that is palpated to reveal two possible lymph nodes measuring 1.2 x 0.8 x 0.4 cm and 1.5 x 0.7 x 0.5 cm. The smaller lymph node is inked black. Both lymph nodes are serially sectioned and submitted entirely in cassette B1.

C. Received in formalin labeled with the patient's name and "female left breast sentinel lymph node #3 time out " is a 3.2 x 1.7 x 0.9 cm aggregate of yellow-tan fibroadipose tissue that is palpated to reveal two possible lymph nodes measuring 0.5 x 0.5 x 0.3 cm and 1.0 x 0.6 x 0.4 cm. The smaller lymph node is inked black. Both lymph nodes are serially sectioned and submitted entirely in cassette C1.

D. Received in formalin labeled with the patient's name and "left female breast sentinel lymph node #4 out at " is a 2.2 x 1.1 x 0.8 cm aggregate of yellow-tan fibroadipose tissue that is palpated to reveal one possible lymph node measuring 0.4 x 0.3 x 0.3 cm. The specimen is serially sectioned and submitted entirely in cassette D1.

E. Received fresh labeled with the patient's information and "female left breast lumpectomy" is a 35 gm lumpectomy specimen measuring 7.0 x 5.5 x 1.0 cm. The specimen has already been inked by the surgeon as follows:

Orange – Lateral

Yellow – Medial

Red – Superior

Blue – Inferior

Green – Anterior

Black – Posterior.

The specimen is serially sectioned from lateral to medial into twelve slices. In slices 6 through 8, there is a 1.2 x 0.7 x 0.6 cm firm white well circumscribed mass that is:

0.1 cm from the anterior margin.

1.8 cm from the posterior margin.

1.5 cm from the superior margin.

2.2 cm from the inferior margin.

2.0 cm from the lateral margin.

1.6 cm from the medial margin.

The remainder of the breast parenchyma is yellow-white adipose tissue. The specimen is entirely submitted as follows:

E1 Slice #1 (lateral margin).

E2 Slice #2.

E3-4 Slice #3 bisected.

E5-7 Slice #4 trisected.

E8-9 Slice #5 bisected.

E10-12 Slice #6 trisected.

E13-14 Slice #7 bisected.

E15-17 Slice #8 trisected.

E18-20 Slice #9 trisected.

E21-22 Slice #10 bisected.

E23-24 Slice #11 bisected.

E25 Slice #12 (medial margin).

The specimen was removed from the patient at and placed in formalin at on and not removed from formalin until

F. Received in formalin labeled with the patient's name and "additional anterior left breast tissue" is a 2.7 x 1.9 x 1.0 cm piece of yellow lobular adipose tissue. The outer surface is inked black. The specimen is serially sectioned to reveal a yellow homogeneous cut surface with no discrete lesion or abnormalities identified. The specimen is entirely submitted in cassettes

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F1-F3. The breast was removed from the patient at , and placed in formalin immediately in the OR and not removed from formalin until on today's date.

If immunohistochemical stains and/or in situ hybridization are cited in this report, the performance characteristics were determined by the in compliance with CLIA'88 regulations. Some of these tests rely on the use of "analyte specific reagents" and are subject to specific labeling requirements by the FDA. Known positive and negative control tissues demonstrate appropriate staining. This testing was developed by the It has not been cleared or approved by the FDA. The FDA has determined that such clearance or approval is not necessary.

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Primary Noted		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	CMS	Date Reviewed: 1/24/14

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